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Kevin James' Unyielding Fight: Surviving Iatrogenic Injuries, Misdiagnoses, and the Emotional Journey to Self-Advocacy

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The Beginning of My Pain Journey

In May 2009, I woke up with sudden pain in my lower right back, along with muscle spasms and stiffness. This made it very hard for me to bend or twist without feeling sharp nerve pain. My doctor advised bed rest, the use of heat packs, and anti-inflammatory medications. Unfortunately, I couldn't tolerate these medications due to severe side effects. Despite months of stretching, walking, and recommended physiotherapy, my pain persisted, and I was eventually told I had developed "chronic pain".



Misdiagnoses and Medical Investigations

Later in 2009, an inconclusive ultrasound led to a diagnosis of lower back pain of “discogenic” origin, with suggestions for hydrotherapy. However, my condition remained unchanged. In early 2012, during my first consultation with a pain specialist, I was diagnosed with right-sided degenerative lumbar osteoarthritis and facet disease. (*Degenerative lumbar osteoarthritis is a condition where the cartilage in the lower back joints wears down over time, leading to pain and stiffness, while facet disease refers to the inflammation or degeneration of the small joints in the back that help stabilize the spine.*)

Along with this, I experienced right hip dysfunction, leg numbness, sciatica, foot neuropathy, and constant sharp nerve pain. From 2012 to 2015, I underwent nine Radio Frequency Ablations (RFAs), yet my pain continued to increase. (*RFAs are minimally invasive procedures that use radio waves to heat and destroy nerve tissue, aiming to reduce or block pain signals. They are commonly used to treat chronic pain conditions, particularly in the back, neck, and joints.*)

In 2016, following new MRI results, a second pain specialist diagnosed me with a probable iatrogenic injury, likely caused by the RFA procedures I had undergone. The MRI showed a tear in the right thoracolumbar fascia, which is a layer of tissue in the lower back. This suggests that one of the muscles in that area is herniated (*pushed out of place or damaged*). It became clear that my injury was worsened by incorrect procedures and physiotherapy exercises that I had been performing in good faith.

Further Complications and Injuries

It was the strong conviction of this new Pain Specialist, that the unusual injury was likely traumatically incurred during one of the many RFA procedures conducted during 2012 to 2015. Up until this point, it had always been impressed upon me that the increasing pain and dysfunction was 100% “nociplastic”, and due to “unhelpful thoughts”, “fear of movement”, and “catastrophizing, but this diagnosis was ultimately incorrect.

In 2015, I suffered an additional “exercise-induced” iatrogenic injury when a partially torn posterior cruciate ligament (PCL) in my right knee was misdiagnosed by both my former GP and a sports physiotherapist. (*The posterior cruciate ligament (PCL) is one of the key ligaments in your knee. It connects your thigh bone (femur) to your*

shin bone (tibia) and helps keep your knee stable, especially when you move or put weight on it.)

They believed it was simply fluid buildup or bursitis beneath the kneecap. As a result of the aggressive exercises they prescribed and their refusal to recommend further diagnostic tests, the PCL is now completely ruptured and non-functional. According to an orthopedic surgeon's evaluation in March 2019, surgery is not an option, as the outcome would likely be worse than the current injury.

The iatrogenic injuries were only confirmed after proper diagnostic procedures. These revealed that my pain was Chronic Secondary Pain, not the suspected Chronic Primary Pain or claims of “non-compliance with exercise” and “opioid hyperalgesia.” (*Opioid hyperalgesia refers to a phenomenon where prolonged use of opioid medications leads to an increased sensitivity to pain, making patients experience more intense pain.*) In hindsight, it's clear that since 2009, I was experiencing chronic secondary pain due to biological, mechanical, and physical issues, worsened by unnecessary procedures and ineffective exercise programs.

Three Key Lessons Learned

Holistic Pain Management: Pain management plans must focus on the whole patient, not just the injury. Every patient's condition is unique and requires individualized care.

The Power of Self-Advocacy: Patients with chronic pain must be educated and empowered to advocate for their own care. Understanding your own condition and pushing for the right treatments is crucial to long-term management.

Empathy in Doctor-Patient Relationships: Physicians must listen to their patients, take detailed histories, and most importantly, show empathy. Research shows that empathy is vital in building trust and creating a successful pain management plan. It fosters a bio-psycho-social approach that patients are more likely to follow and adopt.

WHY?

Recent research indicates empathy is the first vital step in a successful Doctor-Patient relationship,

leading to the development of trust, and a successful bio-psycho-social chronic pain management plan the patient is more likely to adopt and follow.

Coping with Emotional Challenges and Acceptance

In the early days of my chronic pain journey, there were many negative emotions I had to process, including alienation, anger, depression, helplessness, and loss. After losing my job and becoming very socially isolated, I now advocate for individualised treatments and patient-centred care for all chronic pain patients, especially those of us with permanent biological, mechanical, and physical issues, or what is known as Chronic Secondary Pain.

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I also help facilitate a face-to-face Pain Support Group in my local area, as part of the Chronic Pain Australia not-for-profit charity. These monthly sessions help participants to share their experiences with fellow chronic pain patients, leading to the development of better coping strategies and mental health.

One of the most difficult adjustments I have made is around the concept of "Acceptance", where I have learnt to accept and be grateful for the activities I can still perform, rather than focusing negatively on my past career and life.

Current & ongoing pain management strategies:

1. Appropriate use of stable low-dose opioid pain medications.
2. Medical cannabis [CBD].

3. Stretching, graded exercise, and walking daily.
4. Heat packs, meditation, and rest during flare ups.
5. Distraction techniques e.g. music, reading and writing.
6. Practicing gratitude, mindfulness and pacing daily.
7. Cognitive Behavioral Therapy strategies.
8. Acceptance and Commitment Therapy.
9. Professional Allied Health support when required.
10. Social Engagement strategies, e.g. weekly volunteer work.

A Call for Change: Medical Practices and Pain Diagnoses

In conclusion, based on my personal experience, I believe that during initial consultations with medical professionals, whether at local clinics or emergency departments, chronic pain patients are often incorrectly diagnosed with nociplastic pain, including conditions like central or peripheral sensitization, commonly referred to as “brain change” syndromes. Unfortunately, many are also told that any previous injuries have fully healed and there is no remaining tissue damage, which I find counterproductive and, in many cases, comparable to medical gaslighting.

Moving Forward: A Partnership Approach to Pain Management

If chronic pain patients remain undiagnosed, or worse, are misdiagnosed, they may receive harmful treatments, as I did. Many are told their pain is “in the brain” or “all in the head,” which is far from the truth, particularly when symptoms haven’t been fully explored. Going forward, doctors, allied health professionals, and chronic pain patients must collaborate as equal partners in a bio-psycho-social pain management approach. It is crucial that all individuals in pain are believed, validated, and empowered to take control of their care.



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Constantly being in acute pain can be daunting and often a lonely journey. It is a space to rise and fall in stories of pain, healing, and co-existence.
